joint initiative of the health ministry and the department of biotechnology tasked with assessing the cause of the illness via genomic sequencing. Around 12,000 vaccination sites were able to administer 10 million doses a day at the peak of the programme. And the central government funded 1,222 PSA (pressure swing adsorption) plants through PM Cares, to generate 1,750 metric tonnes of captive oxygen daily.

"Covid has given us permanent infrastructural strengthening," says Dr Arora. "We plan to build on it, so that we are prepared for the next variant, the next outbreak, or the next pandemic. If Covid does mutate into something more severe, we are ready."

Much of this additional capacity is now expected to be diverted to other diseases. For instance, the government plans to use all the labs set up for Covid surveillance to monitor other diseases such as cancer, tuberculosis, dengue, monkeypox and tomato flu, among others. The country is also working with South Asian countries such as Bhutan, Sri Lanka and Myanmar to put in place a network of surveillance in the region for pathogens with epidemic potential. "We should never take our eyes off surveillance," says Dr Chauhan. "It is essential to predict and track disease." The present outbreak surveillance of already-known diseases may not be enough, the country also needs to shift focus towards the unidentified viruses. "Bats have been the major source of many recent pandemics caused by airborne viruses, including two coronaviruses—SARS and MERS CoV-2," says Dr Chauhan. "In some parts of the world, there has been an increase in human contact with bats, particularly in Southeast Asian countries. Increasing surveillance of disease outbreaks for emergence of new viruses in these possible hot spot areas as well as in bat populations is the need of the hour."

There is also no guarantee how and when Covid might mutate into a new VoC. Genetic analysis says the probability is much lower today than before and that the virus is becoming more infectious, not more severe. However, with large populations in Africa still unvaccinated and the virus continuing to trigger surges in many countries, the WHO seems unwilling to downgrade Covid's pandemic status anytime soon.

Till then, the government cannot afford to take its eyes off the ball. Booster dose administration



THEN People waiting their turn at a free vaccination camp in Mumbai, Aug. 17, 2021

remains an unfinished project. On October 16, only 23 per cent of the eligible 940 million adults had taken their third precautionary dose. The numbers were slightly better for the above-60 demographic, at 45 per cent. "People think Covid is over, so they aren't coming in for the shot," says Dr Suresh Kumar, the medical director at LNJP. "We are trying through reminders and awareness-building to improve the numbers." There are no plans, though, for a fourth shot as of now. "If we are able to prevent serious disease after three doses and natural immunity, we don't need a fourth dose," says Dr Arora. "Our immediate target is to fully vaccinate everyone over 12 years and precautionary doses for everyone over 18." The government also plans to expand the scope of its CoWIN application—from next month onwards, it will be used to keep track of all routine immunisations.

Experts recommend other targets for the programme to focus on. Among them is designing the right studies to prove whether our current vaccines work to prevent disease or not. "Many of these vaccines are still on restricted use approval, having been licensed on the basis of comparative immunogenicity," says Dr Kang. "If we have the right design, the right data systems, then their clinical evaluation can and should be done." The West has already begun investing heavily in bivalent or strain-specific vaccines. However, a meeting earlier this month of the strategic advisory group of experts

EBOLA AND SARS WERE NOT AS INFECTIOUS OR LONG-LASTING AS COVID AS THEY WERE DEADLIER. ONCE THEIR HOST DIED, THEY COULD NOT TRANSMIT INFECTION